Document Control

Title			
Discharge Prescriptions when the NDDH Pharmacy is Closed Standard Operating Procedure			
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1.0	Jan 2014	Final	Approved by Drugs and Therapeutics Group and published on Bob Jul 14 after some delayed implementation.
1.1	Nov 2016	Revision	Amendment of Table in appendix B to include patient name. Amendment of section 6 to include MDS and blister packs.
1.2	July 2018	Revision	Amended to change registered nurses to registered healthcare professionals. This will allow Nurse Associates to work under the policy. Additional reference added – Use of TTA packs SOP.
2.0	Oct 2018	Final	Final version for approval. Amended to include option on TRAKCARE that doctors should select. Removal of appendix and changed to reference associated standard operating procedure. Removal of need to DATIX on-call pharmacist coming in to dispense a discharge prescription out of hours unless against the criteria stated.
2.1	Nov 2021	Revision	Updated with change in pharmacy availability times at the weekend. Reference to user guide for Pharmacy Tracker to locate items required for TTA packs within the hospital. Addition of details of location of FP10 prescriptions. Criteria for pharmacist to dispense discharge prescriptions out of hours reviewed and updated to define declared OPEL 4.
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1. Introduction

1.1. This Standard Operating Procedure (SOP) seeks to clarify the roles and responsibilities and outline the process of obtaining medications for patients being discharged from the North Devon District Hospital (NDDH) when the NDDH pharmacy is closed.

2. Purpose

2.1. The Standard Operating Procedure (SOP) has been written to:

- Facilitate the discharge of patients out of hours when medications are required
- Ensure that patients are discharged safely when NDDH pharmacy is not open

3. Scope

3.1. This Standard Operating Procedure (SOP) relates to the following staff groups who may be involved in the discharge of patients from the North Devon District Hospital:

- Registered healthcare professionals

4. Location

4.1. This Standard Operating Procedure can be implemented in all clinical areas where competent staff are available to undertake this role.

4.2. Staff undertaking this procedure must be able to demonstrate continued competence as per the organisations policy on assessing and maintaining competence.

5. Procedure

- To ensure safe and timely discharge, wherever possible discharge prescriptions should be written in good time and sent to the pharmacy when it is open. The NDDH pharmacy opening times are:
  - Monday to Friday 08:30 – 17:30
  - Saturday 09:30 – 12:30 and via bleep 017 from 13:00-17:00
  - Sunday 09:30 – 12:30
- In the event that a patient is to be discharged and prior arrangements for medications have not been arranged within pharmacy opening hours, this SOP outlines the options available.

- Out of hours, it is not normally appropriate to discharge patients who have complex medication needs. This includes patients being discharged home on Monitored Dosage Systems (MDS) and blister packs. A senior nurse should be contacted if there are any doubts about discharge. This may then be escalated to the on-call pharmacist through the senior nurse bleep holder if appropriate.
- Please note that controlled drugs cannot be dispensed without pharmacy involvement. However, patient's own controlled drugs can be returned to them on discharge provided they are suitable and appropriate for use.
- A discharge summary must be written for every patient independent of the method of obtaining discharge medication. For discharges out of hours the practitioner writing the summary should select on TRAKCARE "Pharmacy Review Not Required" instead of selecting "Requires Pharmacy Review".
- A print out of all discharge summaries must be signed and filed in the patient's notes. An additional copy should be kept with either the TTA pack log or the FP10(HNC) log as described below.

Medication types that may be present in the patient's bedside locker

Patients must never be given unlabelled medication or medicines with an incorrect label.

Patient's Own Drugs (PODs)

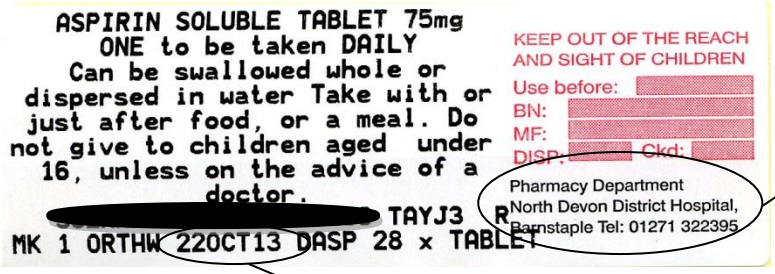
- It is important to distinguish PODs from ward stock and pharmacy issues during the in-patient stay as PODs remain the property of the patient and cannot be kept by the Trust or destroyed without the patient's consent.
- PODs can be identified by a label with the patient's name, issued prior to the date of admission to NDDHT. Any unlabelled medications must be confirmed with the patient as their own. If there is any doubt, consent should be obtained to destroy them, or if not, this should be referred to a senior nurse for advice.
- These medications should be checked against the discharge summary and be given back to the patient if accurate and appropriate and fit for use.

Ward Stock

- Unlabelled medications that have been taken from ward stock cupboards. These must NOT be given to the patient at any time.

Medicines Management Supplies

- Medication issued by the pharmacy department, labelled with the patient's name and instructions for use in anticipation of discharge.



Medicines management supplies will have the NDDH pharmacy details

Medicines management supplies will have been issued since the patient's admission date

- These medications should be checked against the discharge summary and be given back to the patient if accurate and appropriate.

Supply of additional items

- As with all items that are supplied / administered, but with particular care with new items prescribed for patients:
  - A legal prescription must be written.
  - The patient must not be allergic to the contents.
  - The dose, frequency and course length must be safe and appropriate.
- The medication must not significantly interact with any other medications the patient is already taking.
- If these requirements are not met then the prescriber must be contacted.

TTA packs – Refer to the 'Use of TTA packs Standard Operating Procedure'

- To Take Away (TTA) packs are medications that have been pre packed and labelled to allow for discharge when the pharmacy is not open. The labels require only a few annotations to meet the legal requirements for dispensed medications. All of the blank spaces must be completed before giving to the patient.
- The expiry of the TTA pack must be checked to ensure that the pack does not expire before the end of the course.
- TTA packs must always be second checked by a registered professional - nurse, nursing associate, prescriber or pharmacist before giving to the patient.
- Nothing can be added or taken away from a TTA pack. If the quantity in a TTA pack is too much, but the directions are still appropriate, the patient should be advised to return any remaining medication to a pharmacy for safe disposal.
- The Pharmacy Tracker icon, within the standard applications on Novell, can be used by any registered staff member to locate a ward that stocks the required TTA pack. Refer to Pharmacy Tracker user guide within the help and support section on Novell or contact the clinical site manager for advice.

- If there is not a TTA pack to match the prescription it may be appropriate to refer back to the prescriber to see if there is a TTA pack that the prescriber feels is appropriate for the patient. The discharge summary must be updated.

- A log of all TTA pack issues will be kept with the TTA packs and must be completed for all issues, along with a copy of the discharge summary (refer to Use of TTA packs Standard Operating Procedure').

FP10(HNC) Prescription Forms (refer to FP10 Prescription Form Management and Security Standard Operating Procedure)

- FP10 (HNC) prescriptions are located in Emergency Department, Medical Assessment Unit, KGV ward, Bassett ward and Caroline Thorpe ward.
- An FP10(HNC) is an NHS prescription that can be dispensed in any registered pharmacy with an NHS contract. Due to the cost of the prescriptions to the Trust and the limited information available about the patient to the community pharmacist, FP10(HNC)s are not recommended for routine use for medication supply on discharge.
- An FP10(HNC) is controlled stationary and the Prescription Form Management FP10(HNC) SOP must be adhered to at all times. FP10(HNC) prescriptions are only kept in prearranged areas.
- FP10(HNC)s should only be used when the patient needs to go home with an additional item, or two, that are not available as TTA packs. An FP10(HNC) is not a guaranteed supply as the patient must travel to a community pharmacy that has the item(s) in stock.
- The opening times of local community pharmacies can be found at NHS choices http://www.nhs.uk/Service-Search/Pharmacy/LocationSearch/10.
- Patients who do not fit the exemption criteria on the back of the FP10(HNC) will be charged for each item on the FP10(HNC) and should be told so before discharge. Current prescription charges, exemptions and payment options can be found at http://www.nhs.uk/NHSEngland/Healthcosts/Pages/Prescriptioncosts.aspx.
- For the purposes of audit trails related to this SOP, a log of all FP10(HNC) issues will be kept with the prescription pad and must be completed for all issues, along with a copy of the discharge summary.
- The pharmacy receives information regarding all FP10(HNC) prescription dispensed and FP10(HNC) prescribing is monitored. Inappropriate use of FP10(HNC)s will be challenged.

Dispensing of Discharge Prescriptions by On-call Pharmacist

- In the event that these procedures cannot be followed, but discharge medications are necessary, the clinical site manager can be contacted who may authorise contact with the on-call pharmacist.

- Dispensing of discharge prescriptions by the on-call pharmacists is a high risk practice due to lone working and the lack of a pharmacy second check. The usual exceptions for when the on-call pharmacist will come in to dispense discharge medications are:

  - A pharmacy dispensing error which is found at the point of discharge.

  - Palliative care patient who due to unforeseen circumstances requires a rapid discharge to be at home for their final hours.

  - In consultation with the clinical site manager where the Trust has declared OPEL 4 bed status or declared a major incidence as per emergency preparedness plan. This will be for a list of named patients who have a discharge summary already written and ready for pharmacy to process. The on-call pharmacist may assess the clinical need for the discharge prescription and the risks associated with completing a complex discharge prescription (i.e. controlled drugs, blister packs or large quantity of items) out of hours whilst lone working. To ensure patient safety, the on-call pharmacist may determine that it is more appropriate to process the discharge prescription during pharmacy opening hours.

  - Medication is required to be started immediately and there is no TTA pack available for use, no community pharmacy is open to supply medication against an FP10 prescription or it would not be appropriate for the patient/carer to attend a community pharmacy due to infection control measures.

A DATIX report must be made by the on-call pharmacist if they come in to dispense a discharge prescription for any other circumstance other than those listed above.

6. Associated Documentation

6.1. Northern Devon Healthcare NHS Trust Policies for:

Medicines Policy. - https://www.northdevonhealth.nhs.uk/2021/07/medicines-policy/

FP10 Prescription Form Management and Security Standard Operating Procedure - https://www.northdevonhealth.nhs.uk/2019/08/fp10-prescription-form-management-and-control-standard-operating-procedure/

Patients' Own Drugs (PODs) for inpatient setting – Standard Operating Procedure - https://www.northdevonhealth.nhs.uk/2019/05/patients-own-drugs-pods-for-inpatient-setting-standard-operating-procedure/

Use of TTA packs Standard Operating Procedure - https://www.northdevonhealth.nhs.uk/2019/05/use-of-tta-packs-standard-operating-procedure/

Incident Response Plan - https://ndht.ndevon.swest.nhs.uk/wp-content/uploads/2020/10/Incident-response-plan-FINAL-v7.2.pdf